

--SUMMARY--

Decision No. 799/14

28-Jul-2014

B.Alexander - B.Wheeler - A.Signoroni

- Permanent impairment{NEL}

No Summary Available

8 Pages

References: Act Citation

- WSIA

Other Case Reference

- [w3714n]

Style of Cause:

Neutral Citation: 2014 ONWSIAT 1628



WORKPLACE SAFETY AND INSURANCE APPEALS TRIBUNAL

DECISION NO. 799/14

BEFORE:

B. Alexander : Vice-Chair
B. Wheeler : Member Representative of Employers
A. Signoroni : Member Representative of Workers

HEARING:

April 28, 2014 at Toronto
Oral

DATE OF DECISION:

July 28, 2014

NEUTRAL CITATION:

2014 ONWSIAT 1628

DECISION UNDER APPEAL:

WSIB Appeals Resolution Officer (ARO) dated
September 17, 2012

APPEARANCES:

For the worker:

B. Napoleone, Paralegal

For the employer:

Not participating

Interpreter:

M. Chochan, Punjabi

Workplace Safety and Insurance
Appeals Tribunal

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Tribunal d'appel de la sécurité professionnelle
et de l'assurance contre les accidents du travail

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REASONS

(i) Background

- [1] The issue in this appeal is whether the worker has entitlement for loss of earnings (LOE) benefits beyond February 1, 2011, as well as non-economic loss (NEL) awards for the low back and left shoulder.
- [2] The worker, who was born in 1958, began working as a labourer for the employer, a bakery, on March 17, 2008.
- [3] On August 25, 2010, the worker was mopping a floor, when she slipped and fell injuring her low back and left shoulder.
- [4] She laid off work and was paid full LOE benefits from August 26, 2010 to January 3, 2011, when she returned to part time work. She was paid partial LOE benefits until January 17, 2011, when she did not return to work based on advice from her family doctor, Dr. S. Gaur.
- [5] The ARO denied the worker's appeal of a Board decision to deny her NEL awards for her back and left shoulder and LOE benefits beyond February 1, 2011, stating:

While I accept that the worker does have some ongoing problems with her back and shoulders, there is no evidence to support these problems are due to the work injury and the accepted diagnosis of lumbar strain and left shoulder strain. Rather I find the worker's problems are related to her significant pre-existing spinal stenosis with multilevel degenerative disc disease. I find she recovered from this work injury by February 1, 2011 with no evidence of a claim-related permanent impairment.

(ii) The medical evidence

- [6] An x-ray report dated November 21, 2007, noted that the worker had "early degenerative changes in lower lumbar spine with mild narrowing of L5/S1 disc."
- [7] The day after her August 25, 2010 slip and fall, the worker began physiotherapy and a Health Professional's Report dated August 26, 2010, prepared by the worker's physiotherapist, provided a diagnosis of "sprain/strain of chest, lower back, left shoulder, left knee and left ankle."
- [8] A September 29, 2010 report from Dr. Gaur advised that the worker was making a slow recovery from her injuries and was unable to work.
- [9] A Functional Abilities Form (FAF) dated October 21, 2010, also prepared by Dr. Gaur, noted that the worker continued to be unable to return to work due to the injuries to her lower back and left shoulder.
- [10] A report on a November 26, 2010 MRI of the worker's lumbar spine advised:
Multilevel degenerative changes most prominent at L4-L5 with moderate to severe central stenosis. Hypertrophic changes of the posterior elements slightly more prominent on the right than on the left at this level. Disc protrusions at L3-L4 and L5-S1 are symmetric greater off to the left.
- [11] The worker was examined on December 15, 2010, by Dr. K. Minhas, a pain specialist, who reported:

She fell at work and as a result has shoulder, back and hip pain. She has had this pain since August 25, 2010 and the pain is constant. She describes her pain as sharp in nature. She grades her pain 10/10. Pain goes down her legs and buttock. Any activity or movement makes the pain worse [sic] she is unable to sit or stand for long periods of time. Medication such as painkillers partially helps with her pain. There is tingling and numbness associated with both legs, [sic] she also had a slight loss of bladder control and uses diapers at night.

[12] Dr. Minhas administered steroid injections to the worker's lumbar spine.

[13] The worker was referred for an assessment at a Regional Evaluation Centre (REC) and a December 21, 2010 final REC report diagnosed "lumbosacral strain with underlying degenerative changes" and "left shoulder strain."

[14] The "musculoskeletal" section of the report advised:

The insured demonstrated active mobility as follows: flexion to 20 degrees, extension to 20 degrees, bilateral side flexion to 10 degrees and bilateral rotation to 10 degrees, all with reported back pain and all completed in a guarded manner.

Active mobility of the left shoulder was as follows: flexion to 90 degrees, abduction to 85 degrees, both with reported pain. The worker declined to demonstrate the combined movements of hand behind head and hand behind back and internal and external rotation at neutral and at 90 degrees of abduction within normal limits secondary to anticipation of pain.

[15] The report noted that the worker "ambulated with a single point cane in the right hand and with a reciprocal but guarded gait pattern."

[16] The report recommended six additional weeks of "active rehabilitation" and return to work restrictions of increasing hours and increasing weight handling restrictions. The worker was to begin her return to work at the end of week two and was expected to return to regular duties by week nine.

[17] On January 5, 2011, the worker was seen in the emergency department of her local hospital, where she had another lumbar spine MRI and was examined by Dr. K. Grant, an orthopaedic surgeon, who reported:

[The worker] has had a very difficult time since a fall at work back in August. She has claudicant and radicular leg pain and a resent [sic] MRI showing severe stenosis at L4-5. I discussed the possible options of an L4-5 posterior decompression and instrumented fusion with her. I quoted an 80 to 90% chance of improving her leg symptoms with surgery.

[18] In a note dated January 20, 2011, Dr. Gaur stated that the worker was "not fit to work" and in a report dated March 3, 2011, he advised that the worker's back was "shot" and that she was walking with a cane and was unable to return to work.

[19] On April 5, 2011, Dr. Grant reported:

With her current set of symptoms [the worker] is going to be limited in her ability to stand, walk and sit. She should avoid any lifting, bending or twisting. I did explain to her even with a successful surgery I would be probably [sic] avoid physical labour job in the future. I think sedentary work would be best for this woman.

[20] In a report dated December 6, 2011, Dr. Gaur wrote to Mr. Napoleone, as follows:

She never had any shoulder problem in the past. At the time of accident she was working full time with no ongoing shoulder or back problem or complaints.

Despite of [sic] appropriate treatment her condition did not improve and I arranged MRI examination of her back in November 2010. She found [sic] to have severe degenerative disc disease in her spine with spinal stenosis.

In December 2010, she was seen by Dr. K. Minhas a pain specialist for pain control. She did received [sic] some spinal pain block but her pain got worse after the needles and I advised her to stop [sic] pain needles.

Because of worsening of her backache she was seen by Dr. Kevin Grant in Emergency of Scarborough General Hospital on January 5, 2011. Dr. Grant a back and orthopedic surgeon arranged an MRI scan of her back and diagnosed her case [sic] of severe spinal stenosis with multilevel degenerative disc disease. Later he recommended her to get spinal surgery [sic]. Her back surgery was planned on 26 August 2011 but due to lack of family support, [the worker] decided not to proceed with surgery:

Now [the worker] has been suffering from severe backache with pain radiation to both the legs. She also has left shoulder pain. The back pain level is at times 10/10. She takes Tylenol #3 up to 4/ day and Arthrotec 75 mg BID for pain control. The movements in her back are severely restricted. She is unable to walk on toes and heels [sic]. She is unable to squat and walked with stooped posture with a cane. Her left shoulder is sore with painful movements. The impingement test is negative. The latest ultrasound of her left shoulder is normal.

In my opinion [the worker] who was fully functional and working full time prior to work place accident on 25 August 2010, sustained severe trauma to her back and left shoulder.

She has undergone several investigations and treatment including pain medicines, physiotherapy, pain needles and consultations. She is presently severely disabled and unlikely to recover from her present condition. She has been advised to have back surgery but she is not ready for it. She is also depressed because of her illness.

[21] In a memorandum dated July 11, 2011, a Board medical consultant, stated:

The L-spine degeneration was pre-existing and also continued to deteriorate after the workplace injury. The proposed surgery is related to the pre-existing condition and not the workplace injury and therefore it is outside the scope of this claim.

[22] In a further letter to Mr. Napoleone dated March 10, 2014, Dr. Gaur stated:

[The worker] a patient of mine since November 2007 [sic]. In 2007 she presented with bilateral knee pain and lower back pain. The x rays of her lumbar spine done on 21 November 2007 revealed muscle spasm and mild spurring of the 3 lower lumbar vertebrae, with mild narrowing of L5/S1 disc. She was treated symptomatically. She was again seen for backache on 25 August 2008, when complete examination of her back was performed. There was no significant abnormality and a provisional diagnosis of musculus-filamentous backache was made. She was recommended massage therapy.

Apparently [the worker] was working full time in a factory for over 2 years, when she slipped and fell at her work on August 25, 2010 and sustained severe injuries to her shoulder and lower back. She was taken to a nearby hospital in an ambulance and discharged after initial assessment. At the time [sic] accident she was symptom free and was not taking any medicines for pain.

She walks with a cane slowly, with inability to bend forward or backward. The degree of flexion/ extension is limited to less than 15 degree [sic] in her back, with no lateral flexion or rotation.

(iii) The worker's testimony

[23] In her testimony, the worker stated that her job with the bakery was physically demanding, involving the packing, stacking and lifting of baked goods and clean up. Most of the

time she worked while standing. Until her injuries she had lost no time from work and had taken no medication or therapy due to problems with her back or left shoulder.

[24] She had experienced minor back pain for which she saw Dr. Gaur on September 25, 2008, and was prescribed massage therapy. When she began working at the bakery, she had no back or shoulder problems and was in very good health.

[25] On the day of her accident she was asked to clean up water on a floor that had leaked from a freezer. She was mopping it up when she slipped and fell hitting her left shoulder and back on the floor. She had nothing to break her fall.

[26] Prior to her accident, she saw Dr. Gaur last in December 2008 and since her slip and fall, she has seen him almost weekly. She was too scared to proceed with the surgery recommended by Dr. Grant.

[27] She continues to have back and shoulder pain, and physiotherapy and pain killers have provided only temporary relief. She has constant pain, which is at a level of 8 or 9 on a scale of 10 in her back and a level of 7 on a scale of 10 in her left shoulder. She takes Tylenol #3 and Celebrex.

[28] She has been unable to return to work due to her injuries and has difficulty taking a bath or changing her clothes at home. She knows very little English.

(iv) Mr. Napoleone's submissions

[29] In his submissions, Mr. Napoleone noted that prior to the her accident, the worker was able to perform her heavy duties without complaint, had lost no time from work due to injuries and was subject to no restrictions on the performance of her duties.

[30] The August 25, 2010 accident made a significant contribution to the L4-L5 disc protrusion and the stenosis that was seen on the November 26, 2010 MRI.

(v) Law and policy

[31] Since the worker was injured in 2010, the *Workplace Safety and Insurance Act, 1997* (the "WSIA") is applicable to this appeal. All statutory references in this decision are to the WSIA, as amended, unless otherwise stated.

[32] Pursuant to section 126 of the WSIA, the Board stated that the following policy packages, #61 – NEL Entitlement, #223 – LOE Benefits – benefits from December 1, 2010 to July 14, 2011, #227 – Work Integration – from December 1, 2010 to July 14, 2011 and #300 – Decision Making/Benefit of Doubt/Merits and Justice, would apply to the subject matter of this appeal.

[33] We have considered these policies as necessary in deciding the issues in this appeal.

(vi) Conclusions

(a) NEL – low back

[34] *Operational Policy Manual* ("OPM") Document No. 18-05-03, which is included in policy package #61, provides that workers who have a work related permanent impairment are eligible for NEL benefits. The same policy document defines a permanent impairment "as a physical or functional abnormality or loss (including disfigurement) which results from an injury

and any psychological damage arising from the abnormality or loss” which exists after a worker reaches maximum medical recovery (MMR).

[35] The Panel finds that as a result of her August 25, 2010 slip and fall, the worker has a permanent impairment of her lower back and is entitled to a NEL assessment.

[36] According to her testimony and Dr. Gaur, prior to her 2010 injury the worker had no ongoing problems with her lower back and after her 2010 injury her back problems became serious and chronic.

[37] Subsequent to her back injury, the worker was subject to significant limitations in the range of motion of her lower back, which the REC report found to be flexion and extension of 20 degrees and in 2014 Dr. Gaur found to be less than 15 degrees. In his 2010 report, Dr. Gaur indicated that the worker’s back ache was severe “with pain radiation to both legs” and pain levels “at times 10/10.”

[38] The ARO denied the worker a NEL award on the basis that her back problems were related to her pre-existing degenerative conditions and that by February 1, 2011, the worker had recovered from her work related injuries.

[39] There is no evidence that by February 1, 2011, the worker had recovered from her 2010 back injury. Her treating doctors all maintained that her ongoing back symptom, which began with the slip and fall, never resolved. The REC report predicted that with additional “active rehabilitation” the worker would be able to return to modified work but before the worker was to begin her return to work she had an emergency hospital visit due to her back pain, which resulted in the recommendation of back surgery.

(b) NEL – left shoulder

[40] The Panel finds that as a result of her August 25, 2010 slip and fall the worker has a permanent impairment of her left shoulder and is entitled to a NEL assessment.

[41] There is no evidence that prior to her 2010 slip and fall the worker had any problems with her left shoulder.

[42] The slip and fall was severe and the injury to the left shoulder was one of the reasons the worker was unable to return to work. Dr. Minhas described the worker’s left shoulder pain as constant with any activity making it worse and the REC report advised that the shoulder was subject to the range of motion restrictions of flexion to 90 degrees and abduction to 85 degrees.

[43] In his December 6, 2011 report, Dr. Gaur indicated that the worker’s left shoulder continued to be “sore with painful movements.”

(c) LOE benefits from February 1, 2011

[44] The worker began working for the employer as a labourer on March 17, 2008 and there is no evidence that at that time she was suffering from back or shoulder problems or was unable to perform the heavy job she worked at with the employer. Subsequent to March 17, 2008 and prior to August 25, 2010, when she was injured, (a period of over two years), there is also no evidence that the worker was unable to perform her regular duties without losing time from work or that she complained of back or shoulder problems.

[45] The August 25, 2010 slip and fall was a severe accident, in which the worker fell suddenly on a wet floor and was taken to a local hospital by ambulance.

- [46] After her slip and fall her family doctor, Dr. Gaur, and her orthopaedic surgeon, Dr. Grant were both of the opinion that she was unable to return her job due to her compensable injuries.
- [47] We have found that the worker's compensable injuries to her back and left shoulder resulted in permanent impairments for which she is entitled to NEL awards. It is these impairments, along with her limited education and English that prevented her from returning to work as of February 1, 2011.
- [48] Based on the above, we find that the worker is entitled to LOE benefits ongoing from February 1, 2011.

DISPOSITION

[49] The appeal is allowed.

[50] The worker is entitled to NEL assessments for her low back and left shoulder.

[51] The worker is entitled to LOE benefits ongoing from February 1, 2011.

[52] The nature and duration of benefits flowing from this decision will be returned to the WSIB for further adjudication, subject to the usual rights of appeal.

DATED: July 28, 2014

SIGNED: B. Alexander, B. Wheeler, A. Signoroni